FAX TRANSMISSION

From: Cedarbank General Hospital, Health Records Department. Fax 416-555-0177, Tel 416-555-0176
To: Dr. A. Rahman, Northstar Family Health Team, Mississauga. Fax 416-555-0101
Date: 2026-03-10 07:42. Pages: 2 (including this page)
Re: Discharge summary, SAMUELS, B.

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CEDARBANK GENERAL HOSPITAL, MISSISSAUGA
DEPARTMENT OF MEDICINE
DISCHARGE SUMMARY

Patient: SAMUELS, B. DOB: 1948-05-14 (77 y), Male. Health card: [HCN]. Hospital MRN: CB-7731904
Admitted: 2026-03-03 via Emergency. Discharged: 2026-03-09. Length of stay: 6 days
Most responsible physician: Dr. P. Anand, Internal Medicine
Family physician: Dr. A. Rahman, Northstar Family Health Team, Mississauga
Disposition: Home, with support from daughter

MOST RESPONSIBLE DIAGNOSIS

Acute decompensated heart failure with reduced ejection fraction, new diagnosis.

SECONDARY DIAGNOSES

Permanent atrial fibrillation, rate controlled. Type 2 diabetes mellitus. Chronic kidney disease stage 3a. Hypertension, longstanding, now low-normal blood pressure on heart failure therapy.

HISTORY OF PRESENTING ILLNESS

77-year-old man with permanent atrial fibrillation on apixaban, type 2 diabetes and hypertension, presented to the Emergency Department with 5 days of progressive exertional dyspnea, orthopnea (3 pillows), paroxysmal nocturnal dyspnea, bilateral leg swelling and a 4 kg weight gain over two weeks. No chest pain. On arrival: BP 146/88, HR 108 irregular, RR 24, SpO2 89% on room air, JVP 8 cm above the sternal angle, bibasal crackles, pitting edema to the knees. Weight 88.5 kg.

INVESTIGATIONS

NT-proBNP 4200 ng/L. High-sensitivity troponin 38 ng/L, flat on repeat. Creatinine 141 umol/L (eGFR 46), potassium 5.1 mmol/L, sodium 136 mmol/L, hemoglobin 117 g/L, A1c 7.2%, TSH 1.8 mIU/L. Chest radiograph: pulmonary vascular congestion, interstitial edema, small bilateral pleural effusions. ECG: atrial fibrillation, ventricular rate 108, left axis deviation, no acute ischemic change. Transthoracic echocardiogram 2026-03-05: LVEF 30% with global hypokinesis, moderately dilated left ventricle, moderate functional mitral regurgitation, dilated left atrium, RVSP 42 mmHg. Ischemic work-up deferred to the Heart Function Clinic.

HOSPITAL COURSE

Treated with intravenous furosemide with a net diuresis of 5.5 kg. Oxygen weaned by day 3. Guideline-directed therapy was started and titrated as blood pressure and renal function allowed: sacubitril/valsartan 24/26 mg BID on 2026-03-05, increased to 49/51 mg BID on 2026-03-08; carvedilol 6.25 mg BID on 2026-03-06, increased to 12.5 mg BID on 2026-03-08; spironolactone 25 mg daily on 2026-03-07; empagliflozin 10 mg daily on 2026-03-06. Amlodipine was stopped (no role in HFrEF, blood pressure low-normal). Glyburide was stopped because of hypoglycemia risk with reduced oral intake and the addition of an SGLT2 inhibitor. Metformin was reduced to 500 mg BID for eGFR between 45 and 60. Apixaban was continued at 5 mg BID (age under 80, weight above 60 kg, creatinine below 133 umol/L). Potassium peaked at 5.1 mmol/L and was 4.6

mmol/L at discharge. Creatinine 126 umol/L at discharge. Discharge weight 83.0 kg; estimated dry weight 82 to 83 kg.

DISCHARGE MEDICATIONS

1. Sacubitril/valsartan 49/51 mg PO BID (new)
2. Carvedilol 12.5 mg PO BID (new)
3. Spironolactone 25 mg PO daily (new)
4. Empagliflozin 10 mg PO daily (new)
5. Furosemide 40 mg PO daily (new)
6. Apixaban 5 mg PO BID (continued)
7. Metformin 500 mg PO BID (reduced from 1000 mg BID)

Stopped: amlodipine 5 mg daily, glyburide 5 mg BID.

FOLLOW-UP AND INSTRUCTIONS

Family physician within 7 days, with creatinine and electrolytes within 1 week of discharge. Referral sent to the Heart Function Clinic; first appointment expected April 2026. Patient to weigh daily and call the family physician or the clinic if weight rises more than 2 kg in 3 days or symptoms recur. Sodium restriction 2 g daily, fluid 1.5 to 2 L daily. Heart failure teaching completed with the patient and his daughter. Pneumococcal vaccine given 2026-03-08; influenza vaccine previously received this season.

Dictated by: Dr. P. Anand, Internal Medicine, 2026-03-09. Transcribed: 2026-03-10.
cc: Dr. A. Rahman, Northstar Family Health Team; Heart Function Clinic, Cedarbank General Hospital